



DEPARTMENT OF DIAGNOSTIC RADIOLOGY

GUJRANWALA TEACHING HOSPITAL | GUJRANWALA MEDICAL COLLEGE

HOD: Prof. Dr. Mian Waheed Ahmad • Senior Registrars: Dr. Samiya Shahzadi / Dr. Sidra Ghaffar



PATIENT NAME AHMAD	TOKEN / RADIOLOGY ID #7251	AGE / GENDER 64 Y / Male	DATE OF EXAM 12-Sep-2026
MODALITY & PROTOCOL Contrast-enhanced CT of the Abdomen and Pelvis with IV Contrast	REPORTING DATE 12-Sep-2026	REFERRING CLINICIAN Surgical Ward / Gastroenterology	
CLINICAL INDICATION Known case of choledocholithiasis post-ERCP stenting. Presented with acute right upper quadrant pain and localized peritonism.			COMPARISON No previous imaging available for baseline comparison.

TECHNIQUE

ACQUISITION PARAMETERS

Contrast-enhanced CT of the abdomen and pelvis was performed with multiplanar reformations following intravenous contrast administration.

⚠ CRITICAL CLINICAL NOTIFICATION

Acute radiological finding requiring immediate treating team intervention.

Urgent critical notification verbally communicated to General Surgery Registrar on duty at 02:45 PM on 12-Sep-2026 by Radiology Registrar. Read-back confirmed.

FINDINGS

RADLEX ORGAN GROUPING

LOWER THORAX & BASAL LUNGS

- Basal Lung Parenchyma:** Visualised lung bases are clear with no consolidation, mass lesion, or pleural effusion bilaterally.

HEPATOBIILIARY TREE & STENT EVALUATION

- Liver Parenchyma:** Normal in size and overall attenuation with no evidence of focal space-occupying lesion or intrahepatic metastasis.
- Biliary Tree & Stent:** A common bile duct (CBD) stent is in situ; its proximal end extends/migrates into the hepatic parenchyma, while the distal end is situated in the second part of the duodenum with localized extraluminal air extending up to the serosa, concerning for stent malposition with duodenal mural breach.
- Duct Calibre:** The proximal common bile duct is mildly prominent measuring approximately 8.4 mm in diameter; the distal CBD measures 6.0 mm.
- Gallbladder:** Normal in distension and wall thickness with no radio-opaque calculus or pericholecystic collection.

PANCREAS & SPLEEN

- Pancreas:** Normal in size, lobulation, and enhancement. The main pancreatic duct is non-dilated. Peripancreatic fat planes are preserved.
- Spleen:** Normal in size and parenchymal attenuation with no focal splenic lesion.

URINARY TRACT & ADRENALS

- Adrenal Glands:** Bilateral adrenal glands demonstrate normal limb morphology and thickness with no adenoma.
- Kidneys & Ureters:** Both kidneys are normal in size, position, and nephrographic enhancement. Corticomedullary differentiation is preserved. No calculus, hydronephrosis, or focal renal lesion bilaterally. Ureters are non-dilated.
- Urinary Bladder:** Well distended with thin, smooth wall and no intraluminal filling defect or calculus.

GASTROINTESTINAL TRACT & PERITONEUM

- Duodenum & Small Bowel:** Localized extraluminal gas focus adjacent to the second part of the duodenum adjacent to the stent tip. No evidence of diffuse pneumoperitoneum or large bowel obstruction.
- Peritoneum & Ascites:** No free intraperitoneal fluid, collection, or loculated abscess identified in the abdomen or pelvis.
- Lymph Nodes:** A few subcentimeter reactive mesenteric lymph nodes are noted, non-enlarged by standard CT criteria.
- Vascular Structures:** Abdominal aorta, inferior vena cava, celiac axis, and superior mesenteric vessels demonstrate normal caliber and patent flow.

IMPRESSION

- Common bile duct stent in situ with proximal parenchymal migration and distal extension in the second part of the duodenum with localized extraluminal gas, concerning for stent malposition with duodenal mural breach.
- Mild dilatation of proximal CBD (8.4 mm).
- No diffuse pneumoperitoneum or gross free intraperitoneal fluid.
- Urgent communication of these findings to the treating surgical and gastroenterological team is advised.

RECOMMENDATIONS

Immediate surgical and gastroenterological consultation is strongly recommended for urgent evaluation, stent retrieval/repositioning, and management of localized duodenal breach.

PRIMARY REPORTING RADIOLOGISTS

Dr. Sana Fatima — MBBS (WMO Diagnostic Radiology)

Dr. Faisal Maqsood Anwar — MBBS, FCPS-II Resident

Dr. Areeba — MBBS, FCPS-II Resident

REVIEWED & APPROVED BY (CONSULTANTS)

Prof. Dr. Mian Waheed Ahmad

MBBS, FCPS, BSc (Radiology) • Head of Department

Dr. Samiya Shahzadi / Dr. Sidra Ghaffar (Senior Registrars)